

ACR Presidential Address: The American College of Rheumatology in 2025 – The Strength, Resilience, and Importance of Our Rheumatology Community

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Introduction

It is my great pleasure to welcome all of you to Chicago and to the American College of Rheumatology (ACR) Convergence 2025. Whether you are here at this opening session in person or listening to this on demand, the ACR welcomes each of you as we come together at ACR Convergence to learn, share, and celebrate the latest advances in rheumatology.

I want to begin by thanking our Annual Meeting Planning Committee (AMPC), led by Dr Gregory Gardner, for their hard work and dedication. They have developed an outstanding program, with wide-ranging content, innovative formats, and networking options that are designed to meet your professional goals.

Our annual meeting holds a special place for me, as it is where I was first introduced to the ACR. While presentations of the latest science and education represent the foundation of this conference, what keeps me returning every year is the community within rheumatology that is brought together by the ACR. As we open ACR Convergence 2025, it is important, though, that we begin by reflecting on our past year.

The ACR guiding principles and 2025

For many of you, 2025 has represented a period of challenges and uncertainties. During times of challenge, the messages of our professional organizations take on even greater significance. The ACR remains steadfast in upholding its guiding principles (Figure 1), as seen through its mission, which is to empower rheumatology professionals to excel in their specialty, and through unwavering support for its core values. These speak to the necessity of innovation and collaboration in the advancement of rheumatology, the importance of inclusion that recognizes the dignity and belonging of every person and the value of

their perspective, and that holds fast to the true meaning of community. It is through this path of respect for science and for each individual that the ACR demonstrates its commitment to the specialty of rheumatology.

In November 2024, when I had the honor of becoming the 88th President of the ACR, I spoke to the membership of my goal to shine a light on the three mission areas of education, research, and our connections as a community. Little did I know then how important these would become in 2025 and how the ACR's role in supporting these missions would take on a much deeper meaning.

ACR education

Much of my time as an ACR volunteer focused on education such that it was only natural that this would represent one of my focus areas. The rapid advancement of discoveries that benefit patients is one of many rewarding aspects of rheumatology. With these advancements, though, comes the responsibility and privilege of lifelong learning. During this year, I made it a goal to engage in every aspect of the ACR's educational portfolio in support of this vital mission. Through its spectrum of offerings, I saw firsthand how the ACR and Association of Rheumatology Professionals (ARP) provide learning opportunities that allow each person to choose the method and content that addresses their individual needs and preferences. These educational options are delivered through our ACR and ARP journals and publications (*Arthritis & Rheumatology*, *Arthritis Care & Research*, *ACR Open Rheumatology*, and *The Rheumatologist*), podcasts, webinars, and virtual and face-to-face meetings.

Our meetings include differing locations and topics through our Winter Rheumatology Symposium in Colorado; the Pediatric Rheumatology Symposium (PRYSM), which in 2026 will celebrate the 50th anniversary of the first meeting of pediatric

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Figure 1. Guiding principles of the American College of Rheumatology.

rheumatology; and the State-of-the-Art Clinical Symposium (SOTA) held here in Illinois, all of which culminate at the end of our year here at ACR Convergence. Through this wide range of programs, the ACR and ARP strive to be the place where you choose to come to enhance your professional knowledge.

Practice innovation. One aspect recognized by our committees and meeting planners is that education at ACR Convergence is best defined broadly, covering a wide range of topics impactful to every career path. This year saw the launch of the Practice Innovation Summit, a novel advanced programming session designed to provide clinicians, office managers, and interprofessional team members in practice settings with strategies and expert insights to thrive in today's health care landscape. During the 2025 meeting, you will see exciting new program innovations such as this, combined with familiar aspects of ACR Convergence we know and love, including our plenaries, poster hall, and Meet the Professor sessions.

Artificial/augmented intelligence. Another aspect of ACR Convergence, well recognized by our AMPC, is that it needs to include discussion of areas that have emerged in recent times that will impact our field. 2025 saw a dramatic expansion of artificial/augmented intelligence in medicine and health care management, which will continue to grow in the years ahead. This brings exciting opportunities for accelerating research, developing new educational models, and improving efficiencies in clinical practice. With this, though, comes a responsibility to guide its use toward enhancing what must always remain at the core of the practice

of medicine, which is individualized and compassionate care delivered by the provider in making shared decisions with their patient. Maintaining our understanding about this rapidly developing tool provides the most reliable way to ensure it is used in a manner that strengthens and enhances well-reasoned, humanistic medical care.

Addressing medical misinformation. The importance of education has also risen to the forefront in 2025 as we face an environment that has increasingly been filled with medical misinformation that places the safety of our patients at risk. The commitment our rheumatology community has consistently demonstrated toward maintaining evidence-based knowledge and then sharing this knowledge with their patients and others represents an empowering strategy in dispelling inaccuracy and enhancing trust.

Threats to research and barriers to the delivery of care

This evidence base comes to us as a direct result of research, which was the second ACR mission area I sought to focus on during this year. No matter your individual career path, research is critical to each and every one of us because research represents the source of advances that benefit our patients.

2025 has seen many developments that threaten the pace of scientific discovery through impacts on our academic institutions, federal organizations, medical publications, funding levels, and what lines of investigation that funding may be directed toward.

One of the greatest concerns is for our investigators, both those currently engaged in research and the next generation, who represent the future of our field. It is essential to rheumatology that the light of innovation and scientific curiosity be enabled to shine brightly without threat of being dimmed or extinguished.

During this same time, we have also seen barriers placed on the ability of dedicated clinicians to bring those research findings to the care of their patients. This continuum of the pursuit and application of knowledge that improves health represents the very best of humanity that all of us must strive to protect.

The ACR stands side-by-side with investigators and practitioners in driving forth the importance of research and the effective delivery of medical innovations to the care of people with rheumatic disease. This message has been advanced by the ACR through many different avenues.

ACR advocacy. The ACR is extremely fortunate to have a skilled and experienced advocacy team. Their impact has never been more apparent than in 2025, and I want to express my thanks to our advocacy and communications volunteer and staff members for their efforts toward ensuring that the voice of rheumatology is heard throughout all federal, state, and local levels. The ACR has a Washington, DC, office that allows us to be at the forefront of emerging policy decisions and executive actions, to attend briefings, and to request meetings with our legislators and their staff (<https://rheumatology.org/about-acr-advocacy>).

Since the beginning of 2025, the ACR has engaged in issuing over 120 op-eds, letters, media mentions, comments, and public statements (as of September 22, 2025). These have been released by the ACR both as a single organization and in coalition partnerships with other medical associations and state societies, advocating for issues important to rheumatology. The ACR's positions on critical issues are guided and reinforced by our ACR Health Policy Statements developed by our Government Affairs Committee, which are reviewed annually and can be updated at any time to address a newly emerging issue (<https://rheumatology.org/policy-position-statements>). Key policy areas include expanding patient access to rheumatology care, removing barriers to treatment, and increasing support for rheumatology research and training.

In May 2025, the ACR held our annual Advocates for Arthritis meeting in Washington, DC. Over 100 ACR, ARP, and patient delegates attended 123 meetings with bipartisan lawmakers and their staff from 27 states. In these meetings, the ACR brought forward vital messages to our nation's leaders in support of National Institutes of Health-funded research, preserving access to care for Medicare patients, opposing cuts to Medicaid, and urging pharmacy benefit manager reform to lower prescription drug costs and increase access to treatments for patients. During the year, the ACR Executive Committee also met with the US Food and Drug Administration Commissioner and the Deputy Administrator of the Centers for Medicare & Medicaid Services.

As a result of these collective efforts, we have seen a number of successes where the voice of rheumatology has brought about meaningful change. Throughout this time, the ACR advocacy and communications teams have worked hard to keep our membership informed through biweekly messages, as well as postings on the ACR website and social media. We appreciate, though, that there remain challenges that many of you and your colleagues, fellows, and patients continue to face at both state and federal levels. Please be assured that the ACR's advocacy efforts remain steady and ongoing to address the concerns of our membership and patients.

In addition to the actions of our advocacy team, the message of one remains powerful and there are many ways you can individually make a difference. Through the Legislative Action Center, the ACR provides easy ways to contact your legislators about rheumatology issues that matter to you. You can volunteer for ACR advocacy committees; join us in Washington, DC, at our 2026 advocacy event; and support RheumPAC. You can schedule time to speak to the ACR's advocacy team to discuss health care policy issues of concern to you. At ACR Convergence, you will also see signs around the convention center highlighting the ACR's advocacy on current issues and ways in which you can help.

Rheumatology Research Foundation. Another way that you can sustain research growth is through partnership with the Rheumatology Research Foundation, celebrating its 40th anniversary in 2025. The Foundation provides funding that increases the rheumatology workforce and enhances research through project grants and investigator career development awards. As one of largest private funders of rheumatology research, the Foundation's role in accelerating discovery and protecting the future of rheumatology has never been more critical. I hope that you will pursue learning more about our Foundation and join the ACR and me in supporting its vital mission (<https://www.rheumresearch.org>).

Our rheumatology community

My third and final focus area was directed toward enhancing connections between the ACR and its members. The ACR is your community. It has continued to grow and is now over 10,200 members strong.

Whether you are a clinician in practice; a clinical, translational, or basic science investigator; a clinician educator; a fellow; a Master; or a rheumatology professional, you are an essential part of that community who is recognized and respected by the ACR (Figure 2). All of us stand together with one shared goal, which is to improve the lives of those with rheumatic disease.

Having a thriving rheumatology workforce is necessary to achieve that goal. Protecting our workforce means not only growing the number of new rheumatologists and rheumatology professionals but maintaining and supporting those who are already in

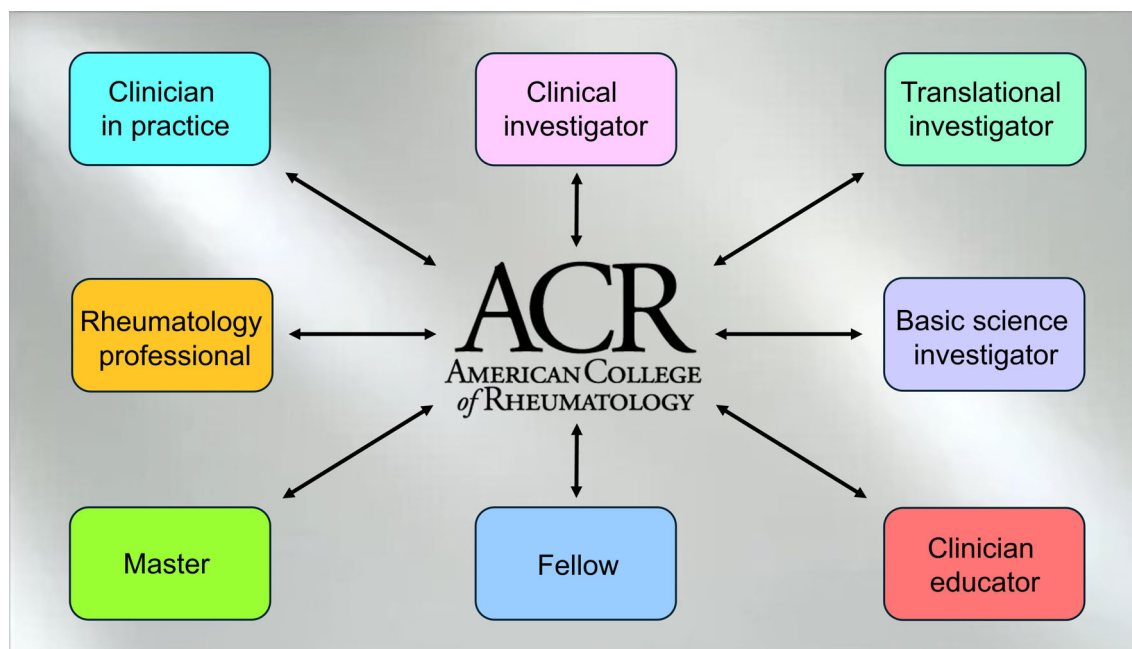


Figure 2. The American College of Rheumatology reflects the many disciplines within rheumatology that together represent one unified community.

our field. Burnout and other forms of occupational distress represent a growing threat in medicine and can affect anyone.^{1–3} In addition to its potential to reduce our rheumatology workforce, occupational distress is important because of its negative effect on the individual experiencing it. When you are part of a community, it means caring about the well-being of those who are within it. It is for these reasons that the ACR chose to invite as our speaker, Dr Tait Shanafelt, a practicing hematologist-oncologist at Stanford University and an internationally recognized thought leader in system approaches to enhance clinician well-being and the impact of burnout in health care, and I am looking forward to his discussion later in this opening session.

An important part of each member's connection to the ACR is in seeing their individual objectives and priorities reflected through the ACR's commitment to our mission. Although this was an aspect of the ACR I had long appreciated, I gained a much deeper understanding of these important links through the President's Corner, which appeared in *The Rheumatologist* (<https://www.the-rheumatologist.org/category/professional-topics/presidents-perspective>). Each of these articles focused on a different aspect of the College and I am grateful to the many ACR, ARP, and Foundation staff and volunteers who helped me review these articles and for the encouragement of Physician Editor Dr Bharat Kumar in making this possible.

The meaning and impact of these connections between the ACR and its member community also became clear from discussions held during the past year with the ACR Board of Directors. At the beginning of each meeting, Board members shared their individual "mission moments," a time meaningful to them when

they saw in action the ACR's mission to empower rheumatology professionals to excel in their specialty. This easily became my favorite part of the meeting, and by the end of the year, seeing how these moments cut across every aspect of the ACR and reflected each discipline within our membership was truly inspiring.

These moments occurred as a direct result of ACR volunteers and staff working together. Providing care to people with vasculitis and participating in the ACR have been the greatest privileges of my career. Being a volunteer with the ACR over the past 31 years has enriched my life. It has allowed me to get to know amazing people I would have never had the opportunity to meet, it enabled me to gain skills I would not have had the chance to learn, and it provided me with an additional way to contribute to this specialty that I care about so deeply. I would encourage all of you to volunteer with the ACR, ARP, or Rheumatology Research Foundation, as your voice is welcomed and needed.

No goals are achieved alone

I have many to thank for the past year. I am grateful to have served under remarkable ACR Past Presidents Drs Kenneth Saag, Douglas White, and Deborah Dyett Desir. I have had the honor of working with fantastic colleagues within the committees and Board of Directors, all of whom have exemplified the true meaning of dedication. I particularly wish to recognize and thank our 2025 Executive Committee: ARP President Dr Adam Goode,

Foundation President Dr Liana Fraenkel, and our ACR leaders Drs William Harvey, Anne Bass, and Angus Worthing. Their experience, guidance, and wisdom were deeply appreciated and essential in 2025.

None of what the ACR achieves, though, would be possible without our incredible staff, all of whom are deeply committed to the ACR and its mission. I wish to express my personal thanks to our Executive Vice President Mr Steven Echard and our entire staff team for everything they do for the ACR, its members, and the patients we serve.

As a child of the 1970s, the song “Dream On” by Aerosmith spoke to me of the future hopes of dreams coming true. Reaching our goals, though, takes a great deal more than dreams. It takes hard work, the strength to learn from those times when things don’t go your way, humility and appreciation for when they do, and the recognition that those dreams were cultivated, supported, and made possible by the actions of others. My standing here would never have occurred were it not for mentors present at every stage of my career who not only taught me rheumatology and clinical research but who most importantly believed in me. These have included Dr James Louie (medical school); Dr Timothy Laing (residency); Drs Rex McCallum, E. William St. Clair, and David Pisetsky (fellowship); Drs Anthony Fauci and Michael Snelker (National Institutes of Health); and Dr Gary Hoffman (Cleveland Clinic). The vital role these physicians played in my life serves as a valuable reminder that mentorship must continue to be sustained, encouraged, and celebrated for its essential place in passing forward both the science and art of medicine.

I also wish to thank my many accomplished collaborators within vasculitis and my own rheumatology family at Cleveland Clinic, particularly my Chair, Dr Abby Abelson. The wonderful people in these communities have sustained me not only during this year but throughout my clinical and research career. There is really nothing better than working together with those you respect who are not just colleagues but also friends.

None of this would, of course, have been possible without my family. My dear late parents, who immigrated here from England, raised me to believe that anything could be possible if I just tried. They taught me the importance of service to others and that the joy of life is not in its accomplishments but in its journey. Finally, I would not be here without the support of my husband, Dr Matthew Bunyard, whom I met during fellowship. His

belief is what sustains me, and the greatest blessing I was ever given was in having him by my side to share life with.

Looking ahead with optimism

ACR Convergence is where rheumatology meets. In between hearing the latest research advances and attending educational sessions, I hope you will take the opportunity to spend time with mentors, fellows, colleagues, and friends, as celebrating these connections is what makes ACR Convergence special. This meeting is more than just a conference—it is a place where all are welcomed and encouraged to engage and to grow.

Our collective commitment to serving people with rheumatic disease fills me with optimism for the years to come. The contributions each of us makes will allow rheumatology to face any challenge through our unity, strength, and resilience. It is in that spirit that I welcome each of you to ACR Convergence 2025, where we meet as one community looking ahead with confidence toward the exciting future of rheumatology.

AUTHOR CONTRIBUTIONS

Dr Langford contributed to at least one of the following manuscript preparation roles: conceptualization AND/OR methodology, software, investigation, formal analysis, data curation, visualization, and validation AND drafting or reviewing/editing the final draft. As corresponding author, Dr Langford confirms that she has provided the final approval of the version to be published and takes responsibility for the affirmations regarding article submission (eg, not under consideration by another journal), the integrity of the data presented, and the statements regarding compliance with institutional review board/Declaration of Helsinki requirements.

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